

geha-coverage-policy-erythropoietin-stimulating-agents.pdf

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Erythropoietin Stimulating Agents:

Aranesp (Darbepoetin alfa), Procrit (Epoetin alfa), Epogen (Epoetin.alfa), Retacrit (epoetin alfa-epbx)

Original Effective Date: 5/2/2022

Revision Date(s): 05/02/2022; 3/18/2023; 2/19/2024; 3/24/2025; 3/16/2026

Review Date: 3/16/2026

Policy type: Medical Necessity.

Line of Business: All G.E.H.A Medical Plans

Authorizations are issued for 6 (six) months, unless the ordering physician requests a different timespan or the patients' unique circumstance or condition supports the medical necessity for a different authorization timeframe. Reauthorization requests are reviewed for efficacy, safety and tolerability.

Preference	Requires Prior Auth	Drug Name	HCPCS Code	Description
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Preferred	Yes	Retacrit	Q5106	Injection, Epoetin alfa-epbx, biosimilar, (retacrit) (for non-esrd use), 1000 units
Preferred	Yes	Aranesp	J0881	Injection, Darbepoetin alfa, 1 microgram (non-esrd use)
Non- Preferred	Yes	Epogen	J0885	Injection, Epoetin alfa, (for non-esrd use), 1000 units
Non- Preferred	Yes	Procrit	J0885	Injection, Epoetin alfa, (for non-esrd use), 1000 units

Members must have documentation of a contraindication, failure, or intolerance to any of the preferred agents prior to approval of a non-preferred product, when clinically appropriate..

Preferred product requirements apply to new starts only.

Step Therapy applies to all overlapping compendia supported indications/regimens..

Criteria applied to all requests:

Hemoglobin values are obtained within 28 days of the date of administration; AND.

Iron stores must be adequate with a transferrin saturation (TSAT) 20% AND a ferritin 100ng/mL within the last 120 days; AND

Other causes of anemia (such as to folate deficiency, B-12 deficiency, iron deficiency, hemolysis,bleeding, bone marrow fibrosis etc.) must be ruled out.

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= G.E.H.A

Indication Specific Criteria:

Chemotherapy Induced Anemia

The member has a diagnosis of non-myeloid, non-erythroid malignancy; AND

The member must be receiving concurrent myelosuppressive antineoplastic therapy; AND

The member's chemotherapy is administered without curative intent; AND

The member has a minimum of 60 days of planned chemotherapy; AND

The member has a hemoglobin level 10g/dL OR Hematocrit is $\leq 30\%$

Anemia secondary to Myelodysplastic Syndrome

The member has a serum erythropoietin level 500 mUnits/mL; AND

The member has a hemoglobin level 11g/dL or hematocrit is 33%

Anemia secondary to Myeloproliferative Neoplasms (MPN) -- Myelofibrosis

The member has a serum erythropoietin level 500 mUnits/mL; AND

The member has a hemoglobin level 11g/dL or hematocrit is 33%

Universal Approval Criteria:

Unless otherwise noted, the review criteria used by OncoHealth to determine medical necessity for anticancer treatments and supportive agents include, but is not limited to:

National Comprehensive Cancer Network@ (NCCN) - use consistent with NCCN recommendations carrying a Category 1 or 2A level of evidence; OR

United States Food and Drug Administration (FDA) labeling - new drugs or regimens (combinations of drugs) consistent with the product labeling; OR

Indications not included in the official FDA labeling or recommended by NCCN (Category 1 or 2A level of evidence) may be considered if determined to be medically necessary per one or more of the following compendia:

O Clinical Pharmacology (Strong For); OR

O Wolters Kluwer Lexi-Drugso (Level A); OR

Other uses of drugs and biologics may be considered medically necessary if supported as safe and effective according to peer-reviewed articles from one of the following journals:

O American Journal of Medicine; Annals of Internal Medicine; Annals of Oncology; Annals of Surgical Oncology; Biology of Blood and Marrow Transplantation; Blood; Bone Marrow Transplantation; British Journal of Cancer; British Journal of Hematology; British Medical Journal; Cancer; Clinical Cancer Research; Drugs; European Journal of Cancer (formerly the European Journal of Cancer and Clinical Oncology); Gynecologic Oncology; International Journal of Radiation, Oncology, Biology, and Physics; The Journal of the American Medical Association; Journal of Clinical Oncology; Journal of the National Cancer Institute; Journal of the National Comprehensive Cancer Network (NCCN); Journal of Urology; Lancet; Lancet Oncology; Leukemia; The New England Journal of Medicine; Radiation Oncology;

O Meeting abstracts and case reports are excluded from consideration;

Non-standard protocols may be approved based on unique clinical circumstances;

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Dose and frequency should be consistent with United States Food and Drug Administration (FDA) labeling, National Comprehensive Cancer Network (NCCN), or indication specific peer-reviewed literature.

In the instance that a request is made for drug(s) that was (were) previously tried (including in the same pharmacologic class or with the same mechanism of action) and such drug(s) was (were) discontinued due to a lack of efficacy the request may be subject to an off-label review for medical necessity unless supported by the NCCN or high quality literature (prospective phase 2 or 3 studies published as full manuscripts in a CMS-supported journal).

Billing

Drug Name	HCPCS Code	Description
Aranesp	J0881 (non-ESRD)	Injection, darbepoetin alfa, 1 microgram (non-esrd use)
Epogen/Procrit	J0885 (non-ESRD)	Injection, epoetin alfa, (for non-esrd use), 1000 units
Retacrit	Q5106 (non-ESRD)	Injection, epoetin alfa-epbx, biosimilar, (retacrit) (for non-esrd use), 1000 units

References

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2. Aranesp. NCCN. Cancer Guidelines and Drugs and Biologics Compendium. Available at:
https://www.nccn.org/professionals/drug_compendium/content
3. Epogen [package insert]. Amgen Inc., Thousand Oaks, CA. Available at:
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https://www.nccn.org/professionals/drug_compendium/content
5. Procrit [package insert]. Janssen Products, LP, Horsham, PA. Available at:
<http://www.janssenlabels.com/package-insert/product-monograph/prescribing-information/PROCrit-pi.pdf>
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7. Retacrit [package insert]. Pfizer Inc., New York, NY. Available at:
<http://labeling.pfizer.com/ShowLabeling.aspx?id=10738>
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https://www.nccn.org/professionals/drug_compendium/content/
9. Referenced with permission from the NCCN Clinical Practice Guidelines in Oncology (NCCN Guidelines). National Comprehensive Cancer Network, Inc. 2026. All rights reserved. Accessed March 10, 2026. To view the most recent and complete version of the guideline, go online to NCCN.org.

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Date	Updates
5/2/22	Policy creation
3/18/23	Updated Indication Specific Criteria; Updated references.
2/19/24	Annual Review; Addition of Step Therapy Table; Updated Universal Criteria; Updated auth length to 6 months; Updated references
3/24/2025	Annual Review including update to universal criteria, disclaimer, authorization language
3/24/2025	Annual review approved at P&T
3/16/2026	Annual review by OH P&T - approved by OH P&T
3/16/2026	Policy adopted by G.E.H.A: document updated for universal accessibility.